

Gastrointestinal Bleeding

History

- Onset of bleeding? How much blood?
- Bleeding upper (hematemesis / melena) or lower / rectal (hematochezia)?
- Associated with pain?
- Medical history – Previous GI bleed, cirrhosis, cancer, anticoagulant therapy (aspirin, Plavix, Coumadin, Pradaxa, Xarelto, Eliquis)
- Alcohol abuse?

Physical

- Abnormal vital signs? – Hypotension, tachycardia, fever?
- Visible blood? How much (estimate)?
- Is the blood dark (coffee ground emesis, black tarry stool/melena, or bright red)?
- Abdominal tenderness or distension?
- Pallor or diaphoresis?

Mild Signs / Symptoms

- Reported rectal bleeding upon bowel movement with no obvious bleeding
- Reported “specks” or “streaks” of blood with vomiting with no obvious blood seen
- Normal vital signs and mental status

Severe Signs / Symptoms

- Obvious blood at scene or on patient
- Obvious rectal bleeding
- Actively vomiting blood
- Hypotension (SBP < 90)
- Tachycardia (HR > 100)
- Altered mental status

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated

EMT/AEMT - Expanded Scope

- **Zofran** 4mg IM or ODT every 5 min x 2 doses if needed (8mg max)
- **NS/LR** 500ml IO bolus x 1

Paramedic

- 2 large bore IV/IO
- **Zofran** 4-8 mg IV/IO for nausea / vomiting
- **NS/LR** 500 mL IV/IO boluses, titrated to
 - Systolic Blood Pressure > 70 mmHg OR
 - Systolic Blood Pressure > 90 mmHg with Heart Rate < 110 beats per min OR
 - ETCO2 > 25 OR
 - Age ≥ 65 yo and SBP > 100 AND HR < 100 beats per minute
- Vasopressor if unresponsive to IVF, titrate to according to above criteria
 - **Epinephrine Push Dose** (1:100,000) 10-20 mcg (1-2 mL) IO/IV every 2-3 minutes PRN (max total 100 mcg / 10 mL) – Consult Medical Director for further doses

